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CHAPTER 2
Overall Principles

Good Psychiatric Management Theory: Interpersonal Hypersensitivity

See Video 1, Psychoeducation
Video 4, Managing Intersession Availability

In Good Psychiatric Management (GPM), the construct of interpersonal hypersensitivity explains borderline psychopathology and informs its interventions (Gunderson 2007; Gunderson and Lyons-Ruth 2008). Borderline personality disorder (BPD) phenomenology shifts dramatically in response to the person's interpersonal context (see Figure 2–1). Within treatment settings, borderline patients frequently vacillate between feeling connected or "held" when they are appealing and compliant, and feeling threatened by perceived hostility or rejection when many of their diagnostically specific feelings and behaviors (e.g., anger, self-harm) become evident. Clinicians who respond supportively will calm their patients, whereas those who respond with anger or withdrawal will activate more distressed and potentially dangerous responses.

Within ongoing relationships, clinicians (or others) can create a "holding environment" (i.e., the patient develops the containing belief that he or she is cared for) by being concerned, consistent, responsive, nonpunitive, curious, nonreactive, and uncertain. Through such experiences of feeling cared for, the borderline patient's negative sense of self can shift toward a sense of self as worthy, good, and lovable (I'm okay, i.e., "not bad," "good enough") and feeling understood (I'm understandable, coherent, not alien). This sense of self as "good enough" can become internalized from a sustained and consistent partnership.

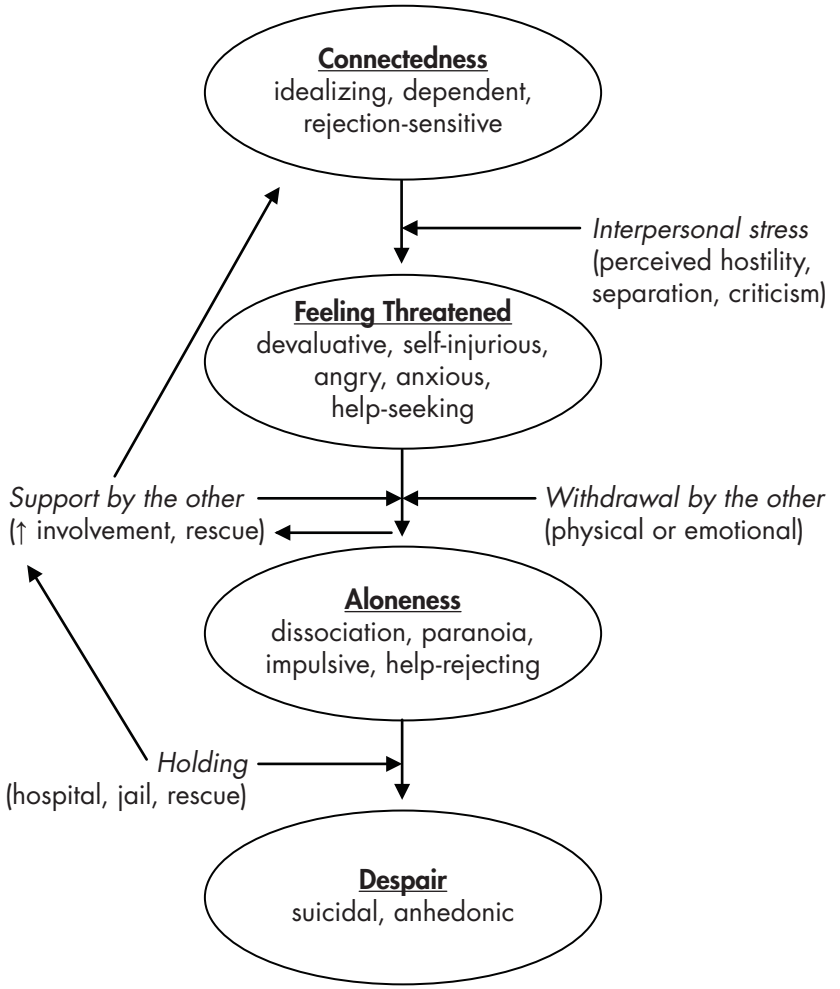


FIGURE 2–1. Borderline personality disorder's interpersonal coherence.

Decreasing borderline patients' experiences of feeling threatened and alone does not usually depend on treaters. For borderline patients who live within stressful situations involving unexpected separations or hostility, their vacillating but basically negative sense of self remains unmodifiable. Some borderline patients will paradoxically cling to stressful relationships or situations, but helping those patients to attain more supportive and stable environments within which they can gain a more positive sense of self can be of substantial and enduring help.

Basic Therapeutic Approach

See Video 2, Diagnostic Disclosure
Video 3, Establishing an Alliance
Video 4, Managing Intersession Availability
Video 5, Managing Safety
Video 6, Managing Anger
Video 7, Managing Medications
Video 8, Managing Safety and Medications
Video 9, Managing Family Involvement

GPM holds that eight basic principles should guide treatment of BPD (see Table 2–1).

1. Offer Psychoeducation

It is helpful to share your knowledge about BPD and even your wisdom about life. Formal psychoeducation about BPD applies to every form of intervention, and examples of how to do this will be evident throughout this book. Informal psychoeducation involves reminding patients of their long-term goals (getting a life), and giving wise advice about romantic or vocational prospects, or even about more mundane things such as transportation and shopping.

2. Be Active, Not Reactive

Borderline patients need to know that you are present. Be responsive, don't accept silences, and raise questions if the patient does not show reactions to what is being said. Being quiet, passive, or minimizing their complaints is experienced as hostile or abandoning. However, do not overreact. Comments about suicide, violence, or other potential disasters require concerned examination. Overreacting with responses such as hospitalizations, medication changes, or consultations will be harmful.

3. Be Thoughtful

Being thoughtful, reflective, cautious, and uncertain are essential parts of containing ("holding") patients. Their world is black or white, all or nothing, and good or bad. Such dichotomous positions are rarely realistic. As explicated and emphasized in Mentalization-Based Treatment (Bateman and Fonagy 2012), you must be comfortable—even insistent—about "not knowing." Your comfort with being uncertain or undecided acts as a container for the patient's polarized black-or-white thinking and provides a model for his or her introjection. Being thoughtful does not mean that you do not need to be active, clear, directive, or authoritative.